

Demo General Hospital - Department of Pathology

This document is fictional and exists only to demonstrate vMTB.

Histopathology report

Patient

Name: [SAMPLE PATIENT] ID: DEMO-0001

Age: 58 years Sex: Male

Referring clinician: Dr. Demo Referrer

Specimen

CT-guided core biopsy, right upper lobe lung mass (3 cores).

Microscopy

Infiltrating adenocarcinoma with acinar and lepidic patterns.

Tumour cells show moderate nuclear pleomorphism. No lymphovascular invasion seen.

Immunohistochemistry

TTF-1: positive Napsin A: positive

p40: negative PD-L1 (22C3): TPS 30%

Diagnosis

Lung adenocarcinoma, right upper lobe, moderately differentiated.

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Molecular profiling report (NGS panel)

Method

Targeted next-generation sequencing, 52-gene solid tumour panel.
Tumour content: 40%.

Clinically relevant findings

EGFR exon 19 deletion (p.E746_A750del) - VAF 22%
TP53 p.R273H - VAF 18%

Negative for

ALK, ROS1, RET and NTRK fusions; KRAS, BRAF V600E, MET exon 14 skipping.

Interpretation

EGFR exon 19 deletion predicts sensitivity to EGFR tyrosine kinase inhibitors.
TMB: 4 mut/Mb (low). MSI: stable.

CT chest and abdomen with contrast

Findings

Right upper lobe spiculated mass, 4.2 x 3.6 cm.

Enlarged right hilar and subcarinal lymph nodes (up to 1.8 cm).

Two hypodense liver lesions (largest 1.4 cm), suspicious for metastases.

No pleural effusion. Bones: no lytic or sclerotic lesions.

Impression

Right upper lobe primary with nodal and liver involvement - clinical stage IVA.

Performance status

ECOG 1 at presentation.